PwC Digital Strategy

Research Quality Review: Statement Classification

Proposed healthcare acquisition for new-market entry | Draft for discussion | 31 October 2026

1. Headline finding

No statement qualifies as Classification 1. The only evidence supplied is the eight stakeholder transcripts, and each is labelled "Fictional practice transcript created for the hackathon; statements require validation." No source documents, data or records about the target were supplied.

Classification 2 applies only to ST-01b, which reports the state of the team's own work. No stakeholder reports first-hand experience of the target. The review covers the 41 statements in the statement register (ST-01 to ST-41); ST-01 is split into two parts because it mixes an opinion with a status report.

2. Classification key

Class	Definition
1	Verifiable fact explicitly supported by supplied evidence
2	Stakeholder-reported experience
3	Opinion or perception
4	Assumption or hypothesis
5	Recommendation
6	Ambiguous or contradictory statement

3. Statement classification

Quotes are verbatim from the transcripts. An ellipsis (…) marks where a longer quote has been shortened. Source codes: S1 Chief Strategy Officer, S2 Commercial / Market Access Director, S3 CMO / Clinical & Product Quality Lead, S4 Regulatory & Healthcare Compliance Director, S5 CIO / Health Data & Cybersecurity Lead, S6 CFO / Healthcare Deal Finance Lead, S7 COO / Supply Chain & Service Delivery Lead, S8 Country / Regional Market Lead.

Statement	Source	Class	What it establishes	What it does not establish	Evidence needed	Safe client wording
ST-01a "The attraction is speed."	**S1**	**3**	S1 sees speed as the reason to consider acquisition.	That a speed advantage exists, or how large it is.	Comparative timelines for acquisition, partnership and organic entry.	"The Chief Strategy Officer sees speed as the main attraction; this has not yet been quantified."

Statement	Source	Class	What it establishes	What it does not establish	Evidence needed	Safe client wording
ST-01b "we have not yet shown which of those advantages the target genuinely owns or whether we could achieve the same objective through a partnership."	**S1**	**2**	S1 reports that ownership of advantages and the partnership option are untested.	Whether the target owns any advantage, or whether partnership would work.	Evidence of ownership and exclusivity; an alternatives assessment.	"The strategy lead confirms the target's advantages and the partnership alternative have not yet been tested."
ST-02 "A broad statement that healthcare demand is growing is not enough to justify this target or its valuation."	**S1**	**3**	S1's view that segment-level evidence is needed.	Actual demand, growth or valuation.	Source-traced segment-level market data.	"The strategy lead considers general demand growth insufficient to support the target or its valuation."
ST-03 "avoid counting the same benefit under both revenue growth and synergy."	**S1**	**5**	S1 recommends avoiding double-counting.	That double-counting has occurred.	Synergy register and revenue build with benefit mapping.	"The strategy lead recommends each benefit be counted once."
ST-04 "If the rationale depends on a market-entry advantage that is not exclusive"	**S1**	**5**	A condition under which S1 would pause.	That the advantage is or is not exclusive.	Evidence on exclusivity.	"The strategy lead would pause if the advantage proves non-exclusive; exclusivity is not yet known."
ST-05 "A source-traced sector scan, a target-specific competitor and market map, validated customer evidence, …"	**S1**	**5**	S1's requested evidence pack.	That any of it exists.	The listed items, with their sources.	"The strategy lead has requested a source-traced evidence pack before the committee decides."
ST-06 "We need to map the actual decision chain for this target."	**S2**	**5**	S2 recommends mapping who decides on adoption.	Who the decision-makers are.	Target business model and buyer map.	"The market access lead recommends mapping the buying decision chain once the target's model is known."
ST-07 "Market growth does not automatically become revenue for one company."	**S2**	**3**	S2's view that growth does not guarantee revenue.	That the target will or will not win customers.	Win/loss, pipeline and competitor data.	"The market access lead cautions that market growth does not by itself mean revenue for the target."
ST-08 "rather than relying only on the target's pitch deck."	**S2**	**5**	S2 recommends independent validation.	That a pitch deck exists or what it claims.	Customer references and independent sources.	"The market access lead recommends validating competitive claims independently."
ST-09 "Customer-level revenue, concentration, contract duration and renewal dates, …"	**S2**	**5**	S2's requested commercial data.	Any actual revenue or contract data.	The listed commercial data.	"The market access lead has requested a defined set of customer and contract data."
ST-10 "The team may assume that existing relationships automatically travel across borders or that our current sales force can cross-sell immediately."	**S2**	**4**	S2 flags two possible assumptions in the thesis.	That the team holds these assumptions, or that they are wrong.	Change-of-control terms; fit with local purchasing; retention evidence.	"The market access lead flags that the thesis may assume relationships transfer and cross-selling is immediate; both need testing."
ST-11 "Marketing material and testimonials alone do not establish clinical benefit."	**S3**	**3**	S3's view on what counts as clinical evidence.	The target's clinical benefit, either way.	Clinical, real-world or performance data.	"The clinical lead notes that marketing material and testimonials do not on their own establish clinical benefit."
ST-12 "Complaints, adverse events or safety incidents where applicable, recalls or field actions, repeat defects"	**S3**	**5**	S3's list of signals to examine.	That any such events occurred.	Complaint, incident, recall and CAPA records.	"The clinical lead recommends reviewing safety and quality signals; none have yet been reviewed."
ST-13 "Yes. A change in ownership, supplier, system, service location, or clinical process can affect availability or continuity."	**S3**	**3**	S3's view that change can threaten continuity (in answer to a question).	That this target is at risk.	Critical products and services; dependency list; continuity plan.	"The clinical lead considers ownership and process change a possible risk to continuity."
ST-14 "how quality is measured, over what period, against which comparator, and who independently verified the data"	**S3**	**5**	S3's questions to test a "high quality" claim.	That management has made the claim, or what the data shows.	Metric definitions, period, comparator, verification.	"The clinical lead would test any quality claim against four questions."

Statement	Source	Class	What it establishes	What it does not establish	Evidence needed	Safe client wording
ST-15 "Any unresolved material safety concern, repeated serious quality finding, unreliable clinical claims, or uncosted remediation plan should be escalated before commitment."	**S3**	**5**	S3's escalation criteria.	That any criterion is met.	Quality audit and safety review.	"The clinical lead has set escalation conditions; whether any apply is not yet known."
ST-16 "We must not assume one country's approval works everywhere."	**S4**	**5**	S4 recommends country-by-country mapping.	Any actual licence position.	Licence and registration map by country.	"The regulatory lead recommends mapping approvals country by country."
ST-17 "We should reconcile management's answers with original records."	**S4**	**5**	S4 recommends checking management answers against originals.	That management's answers are inaccurate.	Inspection reports and regulator correspondence.	"The regulatory lead recommends verifying management's regulatory statements against original records."
ST-18 "Legal specialists should determine which rules apply in each jurisdiction."	**S4**	**5**	S4 recommends legal specialists decide applicable rules.	Which rules apply, or any breach.	Legal analysis by jurisdiction.	"The regulatory lead advises legal specialists determine applicable rules."
ST-19 "The forecast should model the relevant pathway, not simply assume a launch date."	**S4**	**5**	S4 recommends modelling the approval pathway.	Whether the forecast assumes a date.	Regulatory pathway and timeline by country.	"The regulatory lead recommends the forecast reflect the approval pathway."
ST-20 "An unclear licence or registration status, material undisclosed regulator correspondence, serious unresolved quality findings, …"	**S4**	**5**	S4's red-flag criteria.	That any red flag exists.	Licence register; regulator correspondence; legal review.	"The regulatory lead has listed red flags to check before proceeding; none is reported."
ST-21 "We should identify which systems are essential to safe service and which are merely administrative."	**S5**	**5**	S5 recommends classifying systems by criticality.	Which systems exist or are critical.	System inventory.	"The technology lead recommends separating safety-critical from administrative systems."
ST-22 "What personal or health data is collected, why it is used, where it is stored, …"	**S5**	**5**	S5's questions on data governance.	Whether the target holds health data.	Data-flow inventory and legal grounds.	"The technology lead has set out questions on the target's data practices; answers are not yet known."
ST-23 "A policy document alone does not prove controls work."	**S5**	**3**	S5's view on what proves controls.	Whether the target's controls work.	Test results, breach history, remediation evidence.	"The technology lead considers policies alone insufficient to show controls work."
ST-24 "Poor migration or interoperability can interrupt workflows, make records unavailable, break interfaces, or compromise data integrity."	**S5**	**3**	S5's view of integration risk.	That any such event is likely here.	Integration plan; validation and continuity plan.	"The technology lead considers poor migration a possible risk to workflows and records."
ST-25 "Any AI opportunity should be treated as a hypothesis until data rights, workflow fit, safety, oversight and measurable benefits are established."	**S5**	**4**	S5 treats AI upside as unproven.	That any AI use case exists.	Use case; data rights; safety and benefit evidence.	"The technology lead treats any AI opportunity as a hypothesis."
ST-26 "Cyber remediation, licence and vendor costs, data migration, interoperability, …"	**S5**	**5**	S5's list of costs to include.	Any cost amount, or that these costs arise.	Cost estimates after technical review.	"The technology lead recommends costing technology items in the deal case."
ST-27 "If revenue depends on payers, insurers or public reimbursement, examine payer mix, rates, denials, claims ageing and collection cycles."	**S6**	**5**	A conditional recommendation by S6.	Whether the target depends on payers.	Confirmation of the revenue model and payer data.	"The CFO recommends payer analysis if the target depends on payer revenue; that dependence is not yet known."
ST-28 "Reconcile reported revenue to contracts, invoices, cash collections and underlying activity."	**S6**	**5**	S6 recommends revenue reconciliation.	That reported revenue is or is not reliable.	Contracts, invoices, cash records; quality-of-earnings work.	"The CFO recommends reconciling reported revenue to source records."

Statement	Source	Class	What it establishes	What it does not establish	Evidence needed	Safe client wording
ST-29 "liabilities that are not obvious from the headline valuation."	**S6**	**6**	That S6 considers hidden costs possible. The wording presumes both a headline valuation and liabilities, neither defined.	That any valuation or liability exists.	Valuation basis; liabilities review.	"The CFO notes there may be costs beyond the headline price; none has been identified."
ST-30 "The team should show the impact on cash flow and returns rather than just describe risks."	**S6**	**5**	S6 recommends quantified downside modelling.	That a model exists.	Financial model with downside cases.	"The CFO recommends quantifying downside cases in cash flow and returns."
ST-31 "If critical assumptions cannot be validated, the recommendation may need to be conditional or the decision paused."	**S6**	**5**	S6's view that unvalidated assumptions could condition or pause the decision.	That assumptions are unvalidated now.	List of critical assumptions and their validation status.	"The CFO advises the recommendation may be conditional or paused if critical assumptions cannot be validated."
ST-32 "A market forecast does not tell us whether the target can safely deliver the extra demand."	**S7**	**3**	S7's view that capacity needs separate testing.	The target's actual capacity.	Capacity, workforce and bottleneck data.	"The operations lead notes a demand forecast does not show the target can deliver safely."
ST-33 "Identify single-source suppliers, long lead times, cold-chain or special storage needs if relevant, product traceability"	**S7**	**5**	S7's list of supply-chain items to examine, conditional on relevance.	Whether any apply to the target.	Supplier list and dependency analysis.	"The operations lead recommends examining supply dependencies where relevant."
ST-34 "Definitions and historical trends matter more than a one-off snapshot."	**S7**	**3**	S7's view on how to read operational data.	Any operational performance.	Historical data series with definitions.	"The operations lead prefers trends and defined measures to single snapshots."
ST-35 "Changing suppliers, systems, processes or leadership too quickly can disrupt availability, care workflows, service levels or quality."	**S7**	**3**	S7's view of integration pace risk.	That an integration plan is too fast, or exists.	Integration roadmap and stop-authority arrangements.	"The operations lead considers rapid change a possible risk to continuity and quality."
ST-36 "A critical-operations map, business continuity plan, escalation contacts, supplier and service dependency list, …"	**S7**	**5**	S7's Day 1 readiness list.	That any item exists.	The listed documents.	"The operations lead recommends a defined Day 1 readiness package."
ST-37 "Do not equate a large healthcare market with an accessible market."	**S8**	**5**	S8's caution to the committee.	That the target's market is or is not accessible.	Route-to-adoption evidence by country and segment.	"The country lead cautions that a large market is not necessarily an accessible one."
ST-38 "If relationships depend on one founder or a few individuals, we need to know whether those people will remain and whether the relationships are institutional or personal."	**S8**	**4**	A conditional risk.	That such dependency exists.	Relationship mapping; retention arrangements.	"The country lead asks whether key relationships depend on specific individuals; this is not yet known."
ST-39 "Local registration or licensing steps, procurement cycles, reimbursement decisions where applicable, onboarding requirements, …"	**S8**	**3**	S8's view of possible post-close delays.	That any apply, or their duration.	Local timeline analysis.	"The country lead identifies possible local delays that should be built into the timeline."
ST-40 "Would a change of ownership affect trust, contracts, service, or purchasing decisions?"	**S8**	**5**	A question S8 would put to customers.	Customer views, as none were gathered.	Direct customer and partner responses.	"The country lead proposes asking customers whether a change of ownership would affect them."
ST-41 "which assumptions are supported by customer evidence rather than management optimism."	**S8**	**5**	S8 recommends separating customer-backed assumptions from management's.	That management is optimistic.	Customer evidence for each assumption.	"The country lead recommends distinguishing assumptions backed by customer evidence from management's."

4. Reviewer notes

- **Classification calls:** some are judgement calls. Imperative or "should" statements are treated as recommendations (5), including those that also contain an opinion. Statements beginning "can affect" are treated as opinion (3) because no instance is cited.
- **ST-29** is the only item classed as ambiguous (6), because it presumes a valuation and liabilities that the transcripts never define. No statement contradicts another, so no item is classed 6 for contradiction.
- **Safe wording rule:** every wording attributes the view to the named role and says what is not yet known. None presents a stakeholder's view as an organisational fact.
- **Source basis:** all statements come from transcripts labelled as fictional practice transcripts, so none should reach a client as an evidenced finding about a real target.